SILLY TEST
MRN : **A688513638**
Birthday : **1900-02-15** Phone : **2038204947**
Visited on: 2024 Nov 22 12:13 (Age at visit: 124 years) Last edited by: Eric Collins, MD on 2024-11-22

CC

Social anxiety

Psych Syndromes

Patient is doing "ok". She's still pretty anxious about meeting people in groups. She's staying with a family now, and is hesitant to confront them regarding disagreements. SHe gets thoughts like "I'm so dumb, they don't like me, I'm never going to be able to do anything right." She sometimes jumps to worse case scenarios. SHe can't control these worries. She has to fight the urge to leave the situation, and sometimes she does. She easily feels anxious, and "I don't know what to do, I feel frozen". She trusts her mom, and feels unconditional affection toward her, but no one else. No changes in sleep and appetite. No AH, VH. No suicidal or violent ideation.

Patient has no symptoms of anxiety that are impairing or causing excessive distress, and no obsessive or compulsive thoughts/behaviors. Patient is not experiencing acute or chronic symptoms of depression, nor has patient reported a history of symptoms suggestive of mania. Patient denies experiencing traumatic events that cause flashbacks/ nightmares and traumatic events that patient tries to avoid remembering. No history of problematic binging, purging, or restrictive eating habits. No history of disordered sleeping. Additionally, patient is not experiencing hallucinations, and no delusional nor paranoid thoughts were elicited. No cognitive concerns, such as memory or attentional difficulties, were apparent during interview.

Medications

buPROPion HCL XL 150 mg 24 hr tablet, extended release, Take 1 tablet daily

MSE

Appearance: 24-year-old female who appears stated age, good hygiene and grooming, casually dressed, in no apparent distress
Attitude/Behavior: calm & cooperative with evaluation
Psychomotor activity: no psychomotor agitation or slowing
Eye contact: appropriate to conversation
Movements/tremors/mannerisms: no abnormal movements
Speech: normal rate, rhythm, volume and tone
Mood: "anxious"
Affect: constricted, mood-congruent
Thought process: linear and goal-oriented
Thought content: no obsessions, preoccupations, nor delusional content elicited
Perceptions: no auditory or visual hallucinations, not currently responding to internal stimuli
Suicidal/homicidal targets: no suicidal or homicidal ideation
Sensorium/orientation: awake and alert; oriented to person, place and time
Attention: intact to conversation, able to report the days of the week in reverse order
Abstraction: Reports that “Don’t cry over spilled milk” means ___
Fund of knowledge: average
Insight: fair as evidenced by understanding of symptoms
Judgment: fair as evidenced by engagement with treatment

Risk assessment

Patient currently denies suicidal ideation, intent, & plan. In recent months, patient has not considered suicide at any point. Similarly, patient has no history of attempted suicide. Patient presently is not experiencing command auditory hallucinations, global insomnia, intense anxiety, anhedonia, or agitated depression. Substance use is within recommended

Preliminary

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levels, and is stable regarding amount used and frequency of use. Patient has supportive friends/family, a stable job, and a reliance on spirituality. Patient's medical conditions are well-managed and do not negatively impact on quality of life. Patient is future oriented, and reports wanting to live for (3 reasons); no access to firearms. Patient has follow-up appointment with ___ in ___. Due to these reasons, patient's present risk of suicide appears low, and patient is suitable for outpatient treatment at this time.

Psych Intervention

16 minutes were spent providing supportive psychotherapy using reflective listening, empathic validation, and psychoeducation. The patient responded with decreased distress. Topics included, but were not limited to: recent stressors, significant relationships, and recurring cognitive themes.

Assessment

Social phobia (F40.11) Social phobia, generalized modified 22 Nov, 2024

Plan

Social phobia

Medication: Possible risks, benefits, and alternatives were discussed, and patient elects to start propranolol 10mg PRN social anxiety
Psychosocial interventions: Individual psychotherapy recommended, patient will continue sessions with writer
Medical: Labs to be ordered next time
Substance: Patient not using substances
Lifestyle: Patient was informed of the effects that diet, exercise, and good sleep habits can have on social anxiety. Psychoeducation on the Mediterranean Diet, sleep hygiene, and moderate intensity exercise will be given during future sessions.
Return to clinic in 1 month

Miscellaneous

I personally spent 30 minutes on patient care on the date of service. This time included some or all of the following, but not limited to: reviewing previous records, reviewing diagnostic studies and lab results, performing the history and mental status examination, formulating and discussing a patient-oriented treatment plan, counseling the patient and/or caregiver, writing prescriptions, placing orders, documenting the encounter, and coordinating care. Greater than 50% of this time was spent providing direct patient care.

Preliminary